

primary care support requires enhancement. Following the Noto Peninsula Earthquake on January 1, 2024, the Japan Primary Care Association (JPCA) initiated specialized primary care support in Wajima City through December 2024. This report describes comprehensive primary care support activities in a region with over 50% of the elderly population that experienced flooding six months post-earthquake.

Support was provided to an outreach-based primary care clinic where a local physician, an Ishikawa Medical Association's Northern Noto Branch member, served as one of Wajima City's medical coordinators. The clinic provides home medical care, nursing, and rehabilitation services, and operates an NGO offering a "third place" support program for adolescents.

Between February 5 and July 1, 2024, eighteen physicians were dispatched. The team handled 1,487 outpatient visits and 725 home visits with local primary care physicians. This support enabled the local physician to attend 86 regional health-care meetings, including the Wajima City Health and Medical Welfare Coordination Headquarters meetings. Additionally, 29 community activities were supported, including health promotion cafes and programs for high-risk pregnant women. Following severe flood damage in September, which caused one-meter flooding and the loss of outpatient functions, physician dispatch resumed. To enhance sustainability, visiting physicians covered weekend home visits, allowing local staff essential rest periods.

The JPCA aimed to develop support focusing on local healthcare sustainability and human resources. Supporting this crucial medical institution, which functions as the local medical association's disaster coordinator, contributes to maintaining the regional healthcare system's mid- to long-term sustainability.

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Exposure to Severe Weather Caused by Hurricane Harvey and Mortality among Populations of Vulnerable Older Adults

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Introduction: In 2017, unprecedented rainfall from Hurricane Harvey resulted in severe flooding in parts of Texas. While the association between the immediate effects of severe weather on mortality is well established, less is known about long-term mortality and how risk may differ across vulnerable populations, especially those reliant on health services. This study examines the association between severe weather exposure from Hurricane Harvey and mortality among vulnerable populations of older adults.

Methods: This retrospective cohort study used Medicare administrative claims data from beneficiaries aged 65+ who resided in Texas or Louisiana the year before and after Hurricane Harvey. Historic weather data was used to construct

a 4-day measure of ZIP code tabulation area cumulative rainfall and identify locales exposed to high rainfall (above 75mm). Vulnerable older adult populations were identified. Attributable risk and relative risks (RRs) were estimated to quantify the impact of high rain exposure on mortality. Time-to-event analyses estimated associations between exposure to high rain and 12-month mortality.

Results: The study included 1,730,693 beneficiaries; 535,794 (37.5%) were exposed to high rain in Texas, and 264,265 (87.5%) in Louisiana. Beneficiaries with Alzheimer's disease and related dementias (ADRD) exposed to high rain had a 5% higher risk of mortality in fully adjusted models (HR=1.05, 95% CI: 1.02, 1.07) and 1,192 attributable deaths. High rain resulting from Hurricane Harvey was associated with a 13% higher risk of mortality among beneficiaries with end-stage renal disease (ESRD) in fully adjusted models (HR=1.13, 95% CI: 1.04, 1.23) and 85 attributable deaths.

Conclusion: The risk of mortality from exposure to severe weather differs based on the type of vulnerability. Those with an ADRD or ESRD diagnosis, in particular, had elevated mortality rates. High rain exposure had a significantly larger mortality impact on those of lower socioeconomic status. Emergency planning should pay particular attention to these highly vulnerable groups.

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Impact of Armed Conflicts on Continuity and Coordination of Care: A Systematic Review

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Introduction: Continuity of care (COC) and care coordination (CAC) are essential for promotive, preventive, curative, and rehabilitative services within people-centered health systems. However, maintaining COC and CAC is particularly challenging during emergencies and disasters, such as armed conflicts. Armed conflicts result in casualties, population displacement, attacks against healthcare infrastructure and workers, and supply chain disruptions. This systematic review synthesizes evidence on mechanisms and consequences of the impact of armed conflicts on COC and CAC, to provide evidence for future interventions and policies to strengthen healthcare delivery in conflict settings.

Methods: This systematic review follows the Joanna Briggs Institute (JBI) Manual for Evidence Synthesis and PRISMA guidelines. The search strategy targets two key concepts—armed conflicts and continuity/coordinated care—and the resulting strings were run on PubMed MEDLINE, Scopus,